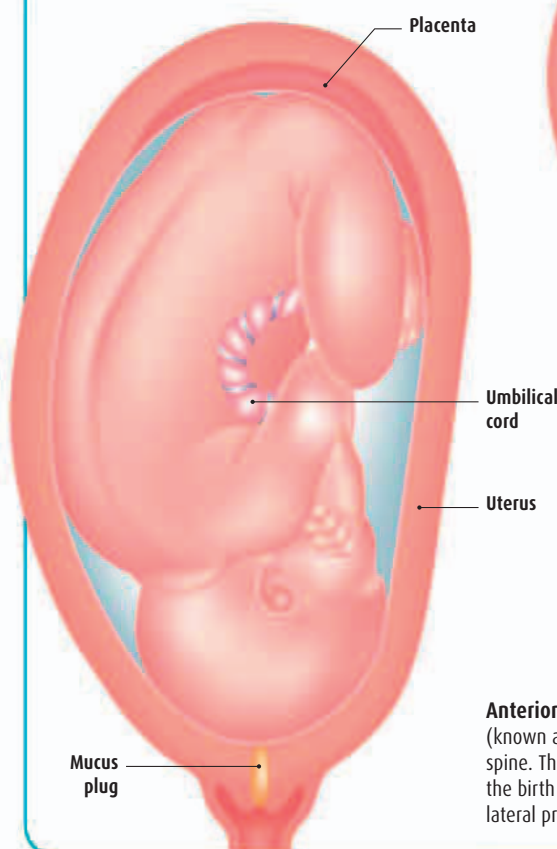


Q My doctor has told me what position my baby is in, but what does this mean and why is it important?

The skeleton of your pelvic region is remarkably flexible, but your baby's position can influence how easy it is for her to get through the pelvic bones and under the pubic arch when the time comes. In an ideal world, your baby's position will allow the smallest point of her head to come out first, and with enough space to bend her neck, and fold in her shoulders.

PRESENTATION POSITIONS

Your baby could be lying in the uterus vertically (known as longitudinal), horizontally (transverse), or diagonally (oblique). If lying vertically, the part the baby "presents" toward your cervix can vary.



Posterior presentation

Also head downward, but your baby's spine is against your spine. From here, the baby needs to make a bigger turn as she descends through the birth canal, so labor can take longer.

Breech presentation

A baby who is breech presents feet or bottom first. There are three possible breech positions—complete, frank, and footling (see p.200).

Anterior presentation Your baby is lying head down (known as cephalic) and her spine is facing away from your spine. The smallest part of your baby's head will push down the birth canal first (this is known as a vertex position). A lateral presentation (spine facing to the side) is also good.

Oblique lie If a baby is lying diagonally across the uterus, the position is called oblique. It's highly unusual for a baby to remain in this position right up until labor. Only one percent of babies will be transverse or oblique.



Transverse lie In the transverse position, your baby is lying straight across your uterus horizontally. The angle means the baby's body doesn't present toward the cervix.

Q How does the position of the baby affect my experience of labor?

Your baby must both descend and rotate through the birth canal to be born (see pp.214–15). In anterior or lateral cephalic presentation (vertex positions), the smallest part of the head leads the way and the baby can maneuver quite easily, helping to keep your labor relatively short and trouble-free. In the posterior position, a wider part of the head presents first and the baby needs to

rotate more to make her descent, making labor progress more slowly. More rarely, it may mean that the baby gets stuck. Labor isn't necessarily more painful in these circumstances, but for your baby it can become distressing. If your doctor feels that labor is taking too long and the baby shows signs of stress, he or she may decide to change the course of your birth plan. Depending upon the circumstances, you may be offered an assisted delivery or, if necessary, a cesarean section. Babies in the transverse and oblique positions during labor are quite likely to need a cesarean section.

37 weeks

Your baby is considered **full-term** at 37 weeks. If you reach 41 to 42 weeks, you are likely to be offered induction to start your labor.